



Izocort Cream

PRODUCT DESCRIPTION: White to off-white cream.

NAME AND STRENGTH OF ACTIVE SUBSTANCES: Isoconazole Nitrate 1% w/w and Diflucortolone Valerate 0.1% w/w.

Other ingredients are Cetostearyl Alcohol, Sorbitan Monostearate, Polysorbate 60, Liquid Paraffin, White Soft Paraffin, Disodium Edetate and Purified Water.

PHARMACODYNAMICS: Isoconazole Nitrate is for use in the treatment of superficial fungal diseases of the skin. It displays a very broad spectrum of antimicrobial action. It is effective against dermatophytes and yeasts, yeast-like fungi (including the causative organism of pityriasis versicolor) and molds, as well as against gram-positive bacteria in-vitro and against the causative organism of erythrasma.

Diflucortolone Valerate suppresses inflammation in inflammatory and allergic skin conditions and alleviates the subjective complaints such as pruritus, burning and pain.

PHARMACOKINETICS

Isoconazole Nitrate: Isoconazole penetrates rapidly into human skin. It is not metabolically inactivated in the skin. Systemic load due to percutaneous absorption is low.

Diflucortolone Valerate: Isoconazole does not influence penetration and percutaneous absorption of Diflucortolone Valerate. Diflucortolone Valerate is partly hydrolysed in the skin to the likewise effective diflucortolone. The portion of the corticosteroid which is percutaneously absorbed is low. Entering the systemic circulation, Diflucortolone Valerate is hydrolysed to diflucortolone and the corresponding fatty acid within minutes.

INDICATION: Initial or interim treatment of those superficial fungal infections of the skin which are accompanied by highly inflammatory or eczematous skin conditions, e.g. in the region of the hands, the interdigital spaces of the feet, and the inguinal and genital regions.

CONTRAINDICATIONS: Tuberculous or syphilitic processes in the area to be treated; virus diseases (e.g. varicella, herpes zoster), rosacea, perioral dermatitis and postvaccination skin reactions in the area to be treated.

Hypersensitivity to the active substances or to any of the excipients.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES: No study have been done.



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ADVERSE EFFECT/UNDESIRABLE EFFECTS:

System Organ Class	Common	Uncommon	Frequency not known
General disorders and administration site conditions	- irritation, - burning	- erythema, - dryness	- pruritus - vesicles
Skin and sub-cutaneous tissue disorders		Skin striae	

DESCRIPTION OF SELECTED ADVERSE REACTIONS: As with other glucocorticoids for topical application, the following local adverse reactions may occur (frequency not known): Skin atrophy, application site folliculitis, hypertrichosis, telangiectasia, perioral dermatitis, skin discolouration, acne, and/or allergic skin reactions to any of the ingredients of the formulation. Systemic effects due to absorption may occur when topical preparations containing glucocorticoids are applied.

Adverse reactions cannot be excluded in neonates whose mothers have been treated extensively or for a prolonged period of time during pregnancy or while lactating (for example, reduced adrenocortical function, immunosuppression).

INTERACTION WITH OTHER MEDICAMENTS: No interaction studies have been performed.

WARNINGS AND PRECAUTIONS: Specific additional therapy is required for bacterial infections of the skin. Izocort should not be allowed to come into contact with the eyes when being applied to the face.

Extensive application of topical glucocorticoids to large areas of the body or for prolonged periods of time, in particular under occlusion, may increase the risk of systemic side effects.

As known from systemic glucocorticoids, glaucoma may also develop from using local glucocorticoids (e.g. after large-dosed or extensive application over a prolonged period, occlusive dressing techniques, or application to the skin around the eyes).

The physician should advise the patients on hygienic measures during the treatment.

If Izocort is applied to the genital regions, the excipients liquid paraffin and white soft paraffin may cause damage of latex products for barrier methods such as condoms and diaphragms used concomitantly, thus impairing their effectiveness.

This medicinal product contains cetostearyl alcohol which may cause local skin reactions (e.g. contact dermatitis).



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PREGNANCY AND LACTATION

Pregnancy: There are no data from the use of Isoconazole Nitrate/Diflucortolone Valerate in pregnant women. In general, the use of topical preparations containing glucocorticoids should be avoided during the first trimester of pregnancy. In particular, treating large areas, prolonged use or occlusive dressings should be avoided during pregnancy. Thus, the clinical indication for treatment with Izocort must be carefully reviewed and the benefits weighed against the risks in pregnant women.

Lactation: It is unknown whether Isoconazole Nitrate/Diflucortolone Valerate are excreted in human milk. A risk to the suckling child cannot be excluded. Nursing mothers should not be treated on the breasts. Treating large areas, prolonged use or occlusive dressings should be avoided during lactation. The clinical indication for the treatment with Izocort must be carefully reviewed and the benefits weighed against the risks in lactating women.

ROUTE OF ADMINISTRATION: Topical

RECOMMENDED DOSAGE: Izocort should be applied twice daily to the diseased areas of skin. Treatment with Izocort must be terminated after regression of the inflammatory or eczematous skin conditions or at the latest after 2 weeks and therapy continued or followed up with a glucocorticoid-free anti-fungal preparation. This applies in particular for use in the inguinal and genital regions.

OVERDOSE AND TREATMENT: Results from acute toxicity studies do not indicate that any risk of acute intoxication is to be expected following a single dermal application of an overdose (application over a large area under conditions favourable to absorption) or inadvertent oral ingestion.

PACKING: 15g in collapsible aluminium tube.

STORAGE: Keep container well closed. Store below temperature 30°C. For external use only. Keep out of reach of children.

RECOMMENDED SHELF-LIFE: 30 months.

MANUFACTURER:

HOE Pharmaceuticals Sdn. Bhd.
Lot 10, Jalan Sultan Mohamed 6, Bandar Sultan Sulaiman, 42000 Port Klang, Selangor, MALAYSIA.

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